



# **ABSTRACT BOOK**

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## Leveraging the National Childhood Cancer Registry to Understand Geographical Health Disparities among Rural Americans

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### Background

Children, adolescents, and young adults living in rural areas of the United States face distinct cancer health disparities compared to their urban counterparts. Although cancer in these age groups is relatively rare, rural youth often experience worse outcomes due to barriers in healthcare access, early diagnosis, and treatment continuity. One major challenge is the limited availability of pediatric oncology specialists and cancer treatment centers in rural regions. Families frequently must travel long distances for care, which can delay diagnosis and treatment initiation, reduce adherence to therapy, and increase financial and emotional stress. Additionally, rural areas often lack supportive services such as psychosocial care, rehabilitation, and survivorship programs tailored to young patients. Cultural factors, including a strong sense of self-reliance and limited health literacy, may further discourage families from seeking preventive care or participating in clinical trials. Data from the CDC and other public health organizations highlight that rural youth are underrepresented in pediatric cancer research, which limits the development of targeted interventions and equitable treatment protocols.

### Purpose

To determine the cancers for which rural residents of the United States have either an over-representation in incidence or mortality (poorer survival) compared to those who live in urban areas, and to identify factors that are associated with increased incidence or poorer survival in rural vs urban areas.

### Methods

We will analyze cancer registry data from the National Childhood Cancer Registry (NCCR) on cancers that occur in children, adolescents, and young adults (CAYA). Data from the NCCR will be analyzed using standard descriptive statistical methods. The comparison of values between rural and urban cancer cases will be tested using t-test for continuous data and chi-square test for categorical data. Kaplan-Meier survival plots will be constructed to visualize differences in survival by rural-urban status. Analyses may be stratified by key variables, e.g., cancer diagnosis, race, ethnicity. Logistic, poisson, or Cox proportional hazards regression models may be used to examine the effects of key variables while controlling for other confounding variable.

### Results

Pending

### Conclusion

Registry data are critical to understanding disparities in cancer incidence/survival and the factors contributing to those differences. NCCR is critical for conducting studies among CAYAs.